

TRAVEL BENEFIT REQUEST

Getting to Medical Appointments — AISH and ADAP

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THERE IS NO GOVERNMENT FORM FOR THIS

The travel benefit is a personal benefit. There is no application to download and no box to tick. It is asked for, in writing, through your worker. That is the main reason so few people know it exists.

This letter does the asking for you. It quotes the provision, sets out which part of it applies to you, and answers in advance the two things most likely to be raised against it.

THE TWO WAYS IN

Section 1 of Schedule 3 of AR 96/2026 allows a travel benefit for travel expenses. Two parts of it cover medical appointments, and they work differently.

Route A — regular access, no travel outside your community required.

Where a client has a health problem that, in a director's opinion, requires regular access to insured health services in Alberta, and is life-threatening or could be permanently debilitating unless those services are provided.

Read that twice, because it surprises people. Route A says nothing about leaving your community. It is about needing to get to care regularly for something serious. Appointments across your own city can qualify.

Route B — travel outside your community.

Where a client must travel outside their community to receive, on a non-emergency basis, insured health services in Alberta.

You can tick both if both apply. The same section also allows a travel benefit for a person who has to accompany you because of your medical condition.

THE TWO THINGS THEY MAY SAY, AND WHY THIS LETTER ANSWERS THEM FIRST

1. That it is available somewhere else. A personal benefit must not be provided where the same or a similar benefit is available from another program or source. So you may be pointed at a hospital travel programme, a low income transit pass, or a specialised transport service. Page 2 asks you to list what you have already tried and what it does not cover, which answers that before it is raised.

2. That you may have to repay it. A personal benefit can carry a condition that you agree to repay it. This letter asks them to state that up front, in writing, before you accept anything. Never agree to a benefit without knowing which kind it is.

There is also an asset test attached to personal benefits. It is worth knowing that before you ask, so it does not come as a surprise if savings are raised.

BEFORE YOU SEND IT

- Send it by email. A phone call leaves no record and the copy in your sent folder is dated proof of when you asked.
- Do not attach medical records. Describe the need and offer to supply documentation if they ask for it.
- Ask for the decision in writing, including the reason if it is refused.
- Do not send this completed letter to the campaign. We cannot submit it for you and your file details should not sit in anyone else's inbox.

SECTION A — YOUR SITUATION

DATE OF THIS LETTER

Maximum 30 characters.

AISH / ADAP FILE NUMBER

Maximum 30 characters.

THE COMMUNITY I LIVE IN

Maximum 40 characters.

WHICH PROGRAM ARE YOU ON?

☐ AISH

☐ ADAP

☐ I have not been told

WHICH ROUTE APPLIES TO YOU? TICK ONE OR BOTH.

☐ ROUTE A — My condition needs regular access to health services and would be life-threatening or permanently debilitating without them.

☐ ROUTE B — I have to travel outside my community for non-emergency insured health services.

☐ I also need someone to accompany me because of my medical condition.

MY HEALTH SITUATION AND WHY THE TRAVEL IS NECESSARY

Plain language. What the condition does to you, what would happen without the appointments, and why getting there is a problem. You do not need medical terms.

Maximum 650 characters.

THE APPOINTMENTS

WHERE I HAVE TO TRAVEL TO

Clinic, hospital, or community. Max 70 characters.

HOW OFTEN

Maximum 40 characters.

HOW I GET THERE NOW

Bus, taxi, driver. Max 40 characters.

WHAT THE TRAVEL COSTS ME, AND WHAT HAPPENS WHEN I CANNOT AFFORD IT

Include appointments you have cancelled, rescheduled, or missed because of the cost. That is the part that carries the request.

Maximum 550 characters.

OTHER SOURCES I HAVE ALREADY CHECKED, AND WHAT THEY DO NOT COVER

Transit passes, hospital travel programmes, specialised transport, community rides. Say what you tried and where it falls short. Write "none available where I live" if that is the case.

Maximum 550 characters.

SECTION B — WHAT I AM REQUESTING

To my AISH / ADAP worker, or to a director:

I am requesting a travel benefit under section 1 of Schedule 3 of the Assured Income for the Severely Handicapped General Regulation (AR 96/2026), for travel to medical appointments. My circumstances are set out in Section A. I request the following.

1. That a travel benefit be provided for my travel to the medical appointments described in Section A, under the route or routes I have identified.
2. Where I have indicated that I require someone to accompany me, that a travel benefit also be provided for that person under section 1(2) of Schedule 3.
3. Written confirmation of the decision. If the request is refused, the reason for the refusal and the provision or policy relied on.
4. If the decision is that a same or similar benefit is available from another program or source, written identification of which program or source, so that I can pursue it.
5. Written confirmation, before I accept anything, of whether repayment will be required as a condition of this benefit, and if so on what terms.
6. Written advice of what documentation you require from my physician or clinic. I will provide it on request.
7. Written advice of any appeal route and time limit that applies to this decision.

I am putting this request in writing so that the date on which it was made is recorded on my file.

ANYTHING ELSE I WANT ON THE RECORD

Optional. Maximum 500 characters.

SIGNATURE (TYPED OR SIGNED)

Maximum 50 characters.

DATE

Maximum 30 characters.

PRINTED NAME

Maximum 50 characters.

CONTACT EMAIL

Maximum 60 characters.

CONTACT PHONE

Maximum 30 characters.

FILE NUMBER

Maximum 30 characters.

Keep a copy and proof of how you sent it. If a benefit is refused, keep the refusal. A written refusal with a stated reason is something that can be reviewed. A verbal no is not.

WHERE TO SEND IT, AND WHAT ELSE TO TRY

Send it to your worker if you have one. If you do not, send it to your regional office and ask them to route it to the worker on your file. Ask for an acknowledgment of receipt.

ALBERTA CONTACT CENTRE	SUPPORTS 1-877-644-9992 • css.ascc@gov.ab.ca 7:30am to 8:00pm, Monday to Friday
CENTRAL ALBERTA	1-833-698-9959 Red Deer, Olds, Drumheller, Camrose, Stettler, Drayton Valley, Lloydminster, Rocky Mountain House, Vermilion, Wainwright, Wetaskiwin, Whitecourt
NORTHERN ALBERTA	1-888-644-1802 Athabasca, Barrhead, Bonnyville, Cold Lake, Edson, Fort McMurray, Grande Prairie, High Level, High Prairie, Hinton, Lac La Biche, Leduc, Peace River, Sherwood Park, Slave Lake, St. Albert, St. Paul, Vegreville, Westlock
SOUTHERN ALBERTA	1-833-382-4081 Lethbridge, Medicine Hat, Brooks, Crowsnest Pass, Pincher Creek, Taber
EDMONTON REGION	780-415-6300 • northzoneaish@gov.ab.ca
CALGARY REGION	1-866-334-6950 or 403-297-8511 • calgaryaish@gov.ab.ca

WHILE YOU WAIT

Ask your clinic whether they have a patient travel or transportation programme. Many hospitals and cancer, renal, and cardiac units do, and their social workers arrange it as routine.

Call 211. They hold the local lists for volunteer driver programmes and community transport, which vary enormously by town and are almost impossible to find on your own.

If you are in Edmonton or Calgary, ask about the low income transit pass and about DATS or Access Calgary. Those are monthly fixes rather than one-off help.

If a missed appointment is putting your health at immediate risk, ask for an emergency benefit as well. That is a different provision and it is also never offered. You have to ask by name.

IF YOU ARE REFUSED

Get it in writing with the reason. Then send it to us. A refusal that does not cite the provision it relies on, or that points at a programme which does not exist where you live, is worth documenting. On its own it is one no. Alongside others it is a record, and a record has to be answered.